

SPECIAL ARTICLE

Executive Summary: Surviving Sepsis Campaign: International Guidelines for Management of Sepsis and Septic Shock 2026

The Surviving Sepsis Campaign (SSC): International Guidelines for Management of Sepsis and Septic Shock 2026 provide guidance on the identification and management of sepsis in adult patients with sepsis. They were developed according to Grading of Recommendations, Assessment, Development, and Evaluations (GRADE) methodology. This executive summary reviews the history, methodology, content, and major changes since the 2021 guidelines.

HISTORY AND SPONSORSHIP OF THE GUIDELINES

The SSC has published guidelines for the management of sepsis and septic shock in 2004, 2008, 2012, 2016, and 2021. The 2026 SSC guidelines are an update from 2021 and focus on evidence published through June 2025.

The guidelines are funded by the Society of Critical Care Medicine and the European Society of Intensive Care Medicine, with methodological support by the Guidelines in Intensive Care Development and Evaluation (GUIDE) group, and endorsement by 24 sponsoring professional societies. There was no industry funding. Panel membership, patient involvement, and conflict of interest management are detailed in the guidelines and the Supplemental Digital Content in the guidelines (1).

METHODOLOGY

Figure 1 in the guidelines (1) summarizes the process for determining the type of statement. We classified graded recommendations as strong (“we recommend”) or conditional (“we suggest”). A strong recommendation indicates that most, if not all, well-informed patients/caregivers in the relevant clinical situation would choose the recommended action or intervention. A conditional recommendation acknowledges that the balance between desirable and undesirable effects may vary depending on patient values, clinical circumstances, or resource availability.

We used standardized language to summarize the findings of evidence syntheses based on effect size (i.e., point estimate) and certainty of the evidence, as recommended by GRADE methodology (2).

We required a minimum 75% response rate and 80% agreement among eligible panelists for all statements and formal remarks. We retained several statements from the 2021 SSC guidelines that remain clinically relevant, accurate, and essential to comprehensive sepsis care. These “carry-over” statements did not undergo updated evidence synthesis but were voted on to ensure continued relevance and accuracy. We also considered the universal applicability of each

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recommendation and comment in the narratives about applicability to different settings, including low resource settings.

Because the diagnosis of sepsis may be uncertain in clinical practice, we developed standardized language for definite, probable, possible, and unlikely sepsis, which are used throughout the SSC guidelines and described in Table 3 in the guidelines (1).

SUMMARY OF 2026 SSC GUIDELINES CONTENT

The 2026 guidelines include 129 statements covering screening and early management (16 statements), infection (25), hemodynamic management (23), respiratory support (14), adjunctive therapies for sepsis (8), additional supportive management (13), goals of care (8), transitions of care (16), and long-term outcomes (6). Most statements (81, 63%) are conditional recommendations, 17 are strong recommendations, 19 are good practice statements, and 11 are statements of insufficient evidence to issue a recommendation.

The 2026 SSC guidelines additionally include 13 “in our practice” statements describing the panel’s practice as determined via independent survey of the panel. These statements are not an endorsement of a specific intervention or treatment approach. Rather, they document how panelists currently approach situations characterized by uncertainty, absence of data, or context-specific clinical variability.

Figures 2 and 3 in the guidelines (1) highlight key recommendations related to antibiotic timing and hemodynamic management, respectively. Table 1 in the guidelines (1) lists all 129 statements and describes how they relate to the 2021 SSC guideline statements. Below we highlight selected statements in the 2026 SSC guidelines that are new, changed, and consistent with the 2021 guidelines.

WHAT IS NEW SINCE 2021

The 2026 SSC guidelines contain 46 statements addressing new questions not covered in the 2021 guidelines. **Table 1** highlights seven new recommendations.

WHAT HAS CHANGED SINCE 2021

The 2026 SSC guidelines contain 38 statements addressing questions that were revisited from the 2021 guidelines. However, only three of these statements differ from prior guidance, as described in **Table 2**.

There are eight statements for which the guidance is consistent with 2021, but the certainty of evidence and/or strength of recommendation has changed. These are presented in **Table 3**.

WHAT IS THE SAME FROM 2021

The 2026 SSC guidelines contain 44 statements that were carried over directly from the 2021 SSC guidelines without an updated evidence synthesis (based on panel assessment that sufficient new evidence to justify a change in recommendation was unlikely). Statements carried over without an updated evidence synthesis are clearly denoted.

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TABLE 1.
Abridged Rationale for Selected New statements

Selected New Statements	Abridged Rationale
3. In acutely ill adults en route to hospital by ambulance or flight, we “suggest” using a standard sepsis screening tool over not using a screening tool. (conditional recommendation, very low certainty evidence)	Sepsis is a time-sensitive medical emergency, and approximately half of patients hospitalized for sepsis arrive via ambulance. Prehospital screening has been associated with improved processes of care. The panel determined that the balance of effects probably favors prehospital, ambulance-based sepsis screening for identifying patients with sepsis and improving the timeliness of sepsis care.
13. For adults with septic shock, we “recommend” an initial MAP target of 65 mm Hg over higher MAP targets. (strong recommendation, moderate certainty evidence) Remark: In practice, it is not feasible to maintain MAP at exactly 65 mm Hg, so a reasonable range (e.g., within 5 mm Hg) should be used. Vasopressors should be titrated to maintain MAP within this range. 14. For adults with septic shock 65 yr old or older, we “suggest” an initial MAP range of 60–65 mm Hg over higher ranges. (conditional recommendation, low certainty evidence)	Consistent with the 2021 guidelines, the 2026 guidelines recommend an initial MAP target of 65 mm Hg. However, the 2026 guidelines include a new remark acknowledging that MAP cannot be maintained at exactly 65 mm Hg, but instead requires a target range, for example, within 5 mm Hg. In a meta-analysis done for the guidelines limited to patients 65 yr old or older, a lower blood pressure target was associated with reduced mortality at longest follow-up. The panel thus determined that the balance of desirable and undesirable effects probably favors permissive hypotension over use of vasopressors to maintain a MAP > 65 mm in adults 65 yr old or older.
21. For adults with definite or probable sepsis and hypotension (i.e., septic shock) and who have an anticipated time to in-hospital medical evaluation of over 60 min, we “suggest” administering antimicrobial therapy in ambulance or flight. (conditional recommendation, very low certainty evidence). Remark: Prehospital antibiotic delivery should be implemented only after having a structured process in place to screen for sepsis in ambulance or flight, as discussed in recommendation 3.	Sepsis is a time-sensitive medical emergency, and approximately half of patients hospitalized for sepsis arrive via ambulance. A meta-analysis of observational studies was uncertain but suggested a possible reduction in mortality with prehospital antibiotics. Meta-analysis of randomized controlled trials also suggested that prehospital antibiotics may reduce 28-d mortality. The panel determined that the balance of effects probably favors prehospital antibiotic administration in patients with sepsis and hypotension and who have an anticipated time to in-hospital medical evaluation of over 60 min. This recommendation aligns with the recommendation to administer antibiotics within 1 hr in patients with septic shock.
28. For adults with sepsis or septic shock without risk factors for anaerobic infection, we “suggest” using an empiric antibiotic regimen without anaerobic coverage. (conditional recommendation, very low certainty evidence) Remark: Agents with anaerobic activity that are needed to cover possible MDR pathogens (e.g., piperacillin-tazobactam, carbapenems) are reasonable to use to provide adequate MDR coverage if alternative agents without anaerobic coverage are inadequate. 29. For adults with sepsis or septic shock with specific risk factors for anaerobic infection, we “suggest” using an empiric antibiotic regimen that includes anaerobic coverage. (conditional recommendation, very low certainty evidence) Remark: Risk factors for anaerobic infection include intra-abdominal or deep seated gynecological/obstetric source of infection, necrotizing soft-tissue infection, head and neck infection, and CNS abscesses or empyema.	The prevalence of sepsis and septic shock due to anaerobic bacteria is low, compared with aerobic bacteria and fungi. Many patients with sepsis or septic shock, particularly of lung or urinary origin, are unlikely to benefit from empiric anaerobic coverage. Observational studies suggest an increased risk of adverse outcomes, including increased mortality, in patients treated with empiric anti-anaerobic antibiotics when no risk factors for anaerobic infection are present. The panel thus issued paired statements suggesting to withhold vs. include anti-anaerobic coverage based on clinical context.

(Continued)

TABLE 1. (Continued)
Abridged Rationale for Selected New statements

Selected New Statements	Abridged Rationale
41. In mechanically ventilated adults with sepsis or septic shock in units with a low prevalence of antimicrobial resistance, we “suggest” using selective decontamination of the digestive tract (SDD). (conditional recommendation, moderate certainty evidence)	Selective decontamination of the digestive tract (SDD) is a preventive infection control strategy consisting of the administration of nonabsorbable, topical antimicrobial agents to the oropharynx and upper gastrointestinal tract (oropharyngeal decontamination), with the administration of a short-term course of broad-spectrum IV antimicrobials in mechanically ventilated patients. SDD is specifically designed to eliminate pathogenic aerobic bacteria while preserving anaerobic gut bacteria essential to digestion and immune function. Meta-analysis of showed a probable reduction in short-term mortality and—counter to concerns—a possible reduction antimicrobial resistance.
89. For adults with septic shock after the acute resuscitation phase, we “suggest” using active fluid removal. (conditional recommendation; very low certainty evidence). Remark: Acute resuscitation refers to escalating doses of vasopressors, ongoing high doses of vasopressors, or needing ongoing volume expansion. Active fluid removal refers to diuretics and, if diuretics are insufficient, ultrafiltration or extracorporeal fluid removal. Factors to be considered when deciding to initiate active fluid removal include: cardiorespiratory function; vasopressor dose; clinical course; peripheral edema; weight; and fluid balance.	Fluid overload in patients with sepsis and septic shock can lead to tissue edema, impaired oxygen delivery, and organ dysfunction and is associated, in observational studies, with increased mortality. The management of fluid balance appears to be important, therefore, particularly in the “evacuation” or “de-escalation” phase of resuscitation. We considered a recent meta-analysis that assessed de-resuscitation strategies. Across all critically ill patients, the pooled analysis demonstrated an uncertain effect of active fluid removal on mortality, with the use of diuretics only having a potentially more favorable effect than the use of diuretics with or without renal replacement therapy. Our suggestion for active fluid removal was influenced by input from patient representatives to the Surviving Sepsis Campaign guidelines, who placed a high value on avoiding edema.

TABLE 2.
Abridged Rationale for Statements that Differ From 2021 Surviving Sepsis Campaign Guidelines

Statements that Differ From 2021	Abridged Rationale
27. For adults with sepsis or septic shock, we “suggest against” using empiric antifungal therapy. (conditional recommendation, low certainty evidence) Remark: Empiric antifungal therapy should be considered on a case-by-case basis in selected patients with sepsis or septic shock and risk factors for fungal infection, including immunosuppression, prolonged use of antibiotics, prolonged hospitalization, and intra-abdominal source of infection.	The 2021 SSC guidelines contained a pair of conditional recommendations that suggested using empiric antifungal therapy in patients at high risk or fungal infection and suggested against using empiric antifungal therapy in patients at low risk of antifungal infection. There are no new data from 2021. The 2026 guidelines streamline the guidance to a single conditional recommendation against using empirical antifungal therapy, recognizing that the majority of patients do not warrant empiric antifungal therapy. However, we have included a remark for case-by-case consideration of empiric antifungal therapy in select circumstances.

(Continued)

TABLE 2. (Continued)**Abridged Rationale for Statements that Differ From 2021 Surviving Sepsis Campaign Guidelines**

Statements that Differ From 2021	Abridged Rationale
42. For adults with septic shock, we “suggest” using either invasive or noninvasive blood pressure monitoring. (conditional recommendation; very low certainty evidence) Remark: Invasive blood pressure monitoring is advised in patients with shock who: require intermediate-to-high dose vasopressors, escalating doses of vasopressor, or multiple vasopressors; are receiving frequent arterial blood sampling; or have noninvasive blood pressure measurements which are inconsistent on repeated assessments.	The 2021 SSC guidelines included a conditional recommendation for invasive blood pressure monitoring. However, considering the very low certainty of evidence, the lack of clear associations with clinical benefit, and concerns regarding equity, patient comfort, and measurement discordance, the panel issued a conditional recommendation supporting the use of either noninvasive or invasive blood pressure monitoring. The multicenter EVERDAC RCT testing a noninvasive strategy to blood pressure monitoring in circulatory failure (i.e., avoidance of arterial line unless pre-specified safety criteria were met) was published after the 2026 SSC guideline recommendations were finalized. EVERDAC found that, among 1010 patients randomized, 28-day all-cause mortality was noninferior in the noninvasive-strategy group (34.3% mortality vs. 36.9% in the invasive strategy, $p = 0.006$ for noninferiority).
45. For adults with sepsis or septic shock, we “suggest” using crystalloids alone over crystalloids with supplemental albumin for fluid resuscitation. (conditional recommendation; moderate certainty evidence). Remark: Use of supplemental albumin may be appropriate for patients who already received large crystalloid volumes or have cirrhosis. Supplemental albumin should be avoided in patients with traumatic brain injury.	The 2021 SSC guidelines included a conditional recommendation for using albumin in patients who had received large volumes of crystalloid. An updated meta-analysis found probably no effect of albumin on prioritized patient-centered outcomes. Given the lack of proven benefit and increased costs, the 2026 guidance has been updated to suggest crystalloids alone over supplemental albumin. We have included a remark to denote specific clinical contexts where albumin may be appropriate on a case-by-case basis.

SSC = Surviving Sepsis Campaign.

TABLE 3.**Statements With a Change in Strength of Recommendation or Certainty of Evidence**

Statements	Change in Certainty or Strength
33. For adults with sepsis or septic shock, we “recommend” using prolonged infusion of beta-lactams for maintenance (after an initial loading dose) over bolus administration. (strong recommendation, moderate certainty evidence)	Upgraded from a conditional to strong recommendation.
36. For adults with sepsis or septic shock, we “recommend” de-escalation of antimicrobial therapy over no de-escalation when a confirmed microbiological diagnosis and susceptibility profile is available. (strong recommendation, very low certainty evidence) Remark: De-escalation involves discontinuing unnecessary antimicrobial therapy or narrowing the spectrum of antimicrobial agents where appropriate.	Upgraded from a conditional to strong recommendation.
44. For adults with sepsis or septic shock undergoing initial resuscitation, we “suggest” using balanced crystalloids over 0.9% saline. (conditional recommendation, moderate certainty evidence). Remark: For patients with sepsis and traumatic brain injury, we suggest using 0.9% saline.	Upgraded from low to moderate certainty evidence.

(Continued)

TABLE 3. (Continued)**Statements With a Change in Strength of Recommendation or Certainty of Evidence**

Statements	Change in Certainty or Strength
49. For adults with sepsis or septic shock, we “suggest” using dynamic measures to guide initial fluid resuscitation over physical examination or static measures alone. (conditional recommendation, low certainty evidence) Remark: Dynamic measures include response to a passive leg raise or a fluid bolus using stroke volume, stroke volume variation, pulse pressure, or pulse pressure variation.	Upgraded from very low to low certainty evidence.
55. For adults with septic shock, we “suggest” using norepinephrine as the first-line agent over vasopressin or angiotensin II. (conditional recommendation) Vasopressin. Low certainty evidence. Angiotensin II. Very low certainty evidence.	Downgraded from strong to conditional recommendation. (Note: Use of norepinephrine as the first-line agent over dopamine, epinephrine, or selegressin remains a strong recommendation.)
57. For adults with septic shock and inadequate MAP levels despite norepinephrine and vasopressin, we “suggest” adding epinephrine. (conditional recommendation, very low certainty evidence). Remark: In settings where vasopressin is not available, epinephrine can be added to norepinephrine alone.	Downgraded from low to very low certainty evidence.
61. For adults with septic shock with persistent hypoperfusion and cardiac dysfunction despite adequate fluid resuscitation and arterial blood pressure, we “suggest” adding dobutamine to norepinephrine or using epinephrine alone. (conditional recommendation, very low certainty evidence). Remark: Data were insufficient to make a recommendation for dobutamine vs. milrinone.	Downgraded from low to very low certainty evidence.
79. For adults with septic shock, we “suggest” using IV corticosteroids. (conditional recommendation, low certainty evidence)	Downgraded from moderate to low certainty evidence.

TABLE 4.**Abridged Rationale for Selected Statements that are Consistent With 2021 Surviving Sepsis Campaign Guidelines**

Selected Statements That are Consistent With 2021	Abridged Rationale
9. Sepsis and septic shock are medical emergencies; treatment and resuscitation should begin immediately. (good practice statement)	This good practice statement was carried over directly from 2021. Sepsis is a leading cause of global mortality, contributing to an estimated 13.7 million deaths annually. In-hospital mortality is higher for patients with sepsis than patients with ST-elevation myocardial infarction. Prompt recognition and treatment reduce mortality.
10. For adults with sepsis-induced hypoperfusion or septic shock, we “suggest” administering at least 30 mL/kg of IV crystalloid in the first 3 hr. (conditional recommendation, low certainty evidence). Remark: Consideration should be given to individual patient characteristics and context when selecting initial fluid volume. Remark: Clinicians prescribing fluids should perform frequent, ongoing reassessment and closely monitor patients to avoid harms of under- or over-resuscitation. Remark: Weight-based fluid volume should be calculated based on actual body weight, or by adjusted or ideal body weight in patients with body mass index >30 kg/m ² (Table 3).	This recommendation is unchanged from 2021. Remarks were added to clarify the importance of ongoing monitoring, consideration of context, and adaptation for patients with high body mass index. Timely, effective fluid resuscitation is crucial for the stabilization of sepsis-induced tissue hypoperfusion in sepsis and septic shock. Although evidence in support of 30 mL/kg stems from observational studies and includes studies evaluating 30 mL/kg as part of a bundle of care practices, there are no new data suggesting a change is needed from prior SSC guidelines. Fluid overload generally occurs with volumes far exceeding 30 mL/kg. However, in patients at higher risk of harms from volume overload, dynamic assessment to guide fluid resuscitation can commence prior to 30 mL/kg (see recommendation no. 49 regarding dynamic measures to guide resuscitation, discussed below).

(Continued)

TABLE 4. (Continued)**Abridged Rationale for Selected Statements that are Consistent With 2021 Surviving Sepsis Campaign Guidelines**

Selected Statements That are Consistent With 2021	Abridged Rationale
16. For adults with possible, probable, or definite septic shock, we “recommend” administering antimicrobial therapy immediately, ideally within 1 hr of recognition. (strong recommendation, very low certainty evidence) 17. For adults with probable or definite sepsis without shock, we “recommend” administering antimicrobial therapy immediately, ideally within 1 hr of recognition (strong recommendation, very low certainty evidence) 18. For adults with possible sepsis without shock, we “suggest” a time-limited course of rapid investigation and if concern for infection persists, the administration of antimicrobial therapy within 3 hr from the time when sepsis was first suspected. (conditional recommendation, very low certainty of evidence)	These recommendations are unchanged from 2021 aside from minor wording changes for clarity. Early administration of appropriate antimicrobials is the most effective initial intervention to reduce mortality in patients with sepsis or septic shock, along with fluid resuscitation. Given the high risk of death with septic shock and the more consistent and stronger association of antimicrobial timing and short-term mortality in patients with septic shock, the panel issued a strong recommendation to administer antimicrobials immediately (within one hour) in adults with possible, probable, and definite septic shock and in adults with probable or definite sepsis. For adults with possible sepsis without shock, where the diagnosis of bacterial infection is less clear, the panel issued a conditional recommendation for a rapid (within 3 hr) assessment of infectious and noninfectious etiologies of illness. This assessment may include additional history, clinical examination, and diagnostic testing to determine whether antimicrobials should be administered. Antimicrobials should be administered as soon as bacterial infection appears to be the most likely etiology of the patient's illness and after no more than 3 hr if a substantial concern for bacterial infection persists.
49. For adults with sepsis or septic shock, we “suggest” using dynamic measures to guide fluid resuscitation over physical examination or static measures alone. (conditional recommendation, low certainty evidence) Remark: Dynamic measures include response to a passive leg raise or a fluid bolus using stroke volume, stroke volume variation, pulse pressure, or pulse pressure variation.	This recommendation is unchanged from 2021. Patients with sepsis or septic shock often require additional IV fluid following an initial 30 mL/kg fluid resuscitation. However, additional resuscitation fluid must be balanced against the risk of fluid accumulation and potential harm associated with fluid overload. To avoid over-resuscitation or under-resuscitation, fluid administration beyond the initial resuscitation phase should be guided by careful assessment of intravascular volume status, organ perfusion, and fluid responsiveness. In a meta-analysis done for these SSC guidelines, fluid management guided by dynamic measures likely reduces mortality and may result in a slight increase in ventilation-free days.

SSC = Surviving Sepsis Campaign.

The 2026 SSC guidelines additionally contain 39 statements that were revisited from 2021 SSC guidelines, of which 7 are unchanged and 29 are consistent with the 2021 recommendations but have revised wording and/or a new remark to refine or clarify guidance. **Table 4** highlights recommendations that are consistent with the 2021 SSC guidelines.

CONCLUSIONS

This executive summary describes the contents of the 2026 SSC guidelines and highlights selected new, revised, and unchanged statements that are most relevant to daily practice. The guidelines document

contains all 129 statements and narratives for all statements informed by a new or updated evidence synthesis.

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Drs. Prescott and Antonelli served as guidelines co-chairs. Drs. Alhazzani and Möller served as methods co-leads. Drs. Alshamsi, Azevedo, Belley-Cote, De Waele, Derde, Dionne, Evans, Gershengorn, Hodgson, Honarmand, Kesecioglu, McIntyre, Mer, Nunnally, Oczkowski, and Rochwerg served in guidelines leadership roles. Drs. Akinola, Akuamoah-Boateng, Alberto, Angus, Arabi, Azoulay, Cecconi, Convocar, De Pascale, Doi, Du, Egi, Elie-Turenne, Ferrer, Fox-Robichaud, French, Freund, Gong, Hale, Hammond, Hashmi, Heunks, Iwashyna, Jacob, Klompas, Kwizera, Leeies, Lejniaks, Levy, Machado, Maia, Masur, Maves, McGloughlin, McPeake, Mohr, and Myatra served as guidelines panelists. Drs. Ostermann and Peake served as Surviving Sepsis Campaign leaders.

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